

Follow-up antenatal visit continued:

Assess	When to assess	Note
Abdomen	Every visit	- Measure and plot SFH¹ according to gestation on SFH growth chart. - Assess growth by looking at pattern of growth over time and where measurement falls in relation to percentile lines: - If < 28 weeks and measurement > 90th centile or multiple pregnancy likely, refer. - Refer/discuss for likely poor fetal growth if: - Two <i>consecutive</i> SF measurements below 10th percentile (or <i>three separate</i> SF measurements below 10th percentile) - Three <i>consecutive</i> SF measurements remain the same (or <i>two consecutive</i> SF measurements, taken at least 6 weeks apart, remain the same). - SF measurement less than measurement recorded 2 visits previously. - If ≥ 34 weeks: palpate presenting part. If breech or transverse lie suspected, reassess at ≥ 38 weeks. If still suspected, refer.
Vaginal discharge	Every visit	- If abnormal discharge, treat ↗ 49. - If watery discharge with history of a sudden gush of clear or pale fluid from vagina, and no contractions, suspect prelabour rupture of membranes ↗ 159.
BP	Every visit	- If BP ≥ 160/110, manage and refer urgently → 159. - If ≥ 140/90, lie patient on left side for at least 1 hour, then repeat BP: - If repeat BP ≥ 150/100, refer same day ↗ 159. - If repeat BP < 150/100, check urine dipstick for protein: - If ≥ 1+ proteinuria, refer same day. - If no proteinuria but headache, blurred vision or severe abdominal pain, treat for severe pre-eclampsia ↗ 159. - If no proteinuria, educate about warning signs (persistent headache, blurred vision or abdominal pain), advise to rest/reduce workload and review in 1 week: - If BP at review ≥ 140/90, arrange same day doctor/advanced midwife review: treat for gestational hypertension ↗ 163 and review weekly. If proteinuria/symptoms develop, refer urgently ↗ 159. - Refer all at 38 weeks for hospital delivery.
Urine dipstick: test clean, midstream urine	Every visit	- If leucocytes and nitrites in urine treat for likely complicated urinary tract infection ↗ 59. - If proteinuria: - If trace or 1+ proteinuria with normal BP, reassess at next antenatal visit. If BP raised, manage above. - If ≥ 2+ proteinuria, repeat dipstick on a new urine specimen: if still ≥ 2+ proteinuria, discuss/refer. - If glucose in urine, check diabetes risk.
Diabetes risk	If risk factor ² : 26 weeks	Give unfasted patient oral glucose 75g in 250mL water upon arrival at clinic. Check glucose after 1 hour: if ≥ 7.8, arrange further fasting test at high risk clinic.
Haemoglobin (Hb)	- Between 28 and 32 weeks - At 36 weeks - If patient pale - If Hb < 10: 1 month after treatment started	Give iron according to Hb ↗ 163. Refer if: - If Hb < 6, or Hb 6-7.9 <i>with</i> symptoms (dizzy, pulse > 100, difficulty breathing at rest): refer same day to regional hospital. - If Hb 6-7.9 without symptoms: refer within 1 week to next level of care clinic. - If Hb 8-9.9 and Hb is not improving after 1 month of treatment: refer within 1 week to next level of care clinic. - If Hb < 10 at ≥ 36 weeks: refer to next level of care clinic and arrange delivery at hospital.
If Rh negative: anti-D antibodies	Send Coombs test to check for antibodies at 26, 34 and 38 weeks	- If Coombs positive, refer. - If Coombs negative, give anti-D immunoglobulin 100mcg IM after delivery/miscarriage preferably within 72 hours, up to 7 days later.
Syphilis	20, 26, 30, 34, 38 weeks and at delivery	- Use fingerprick test, as result immediately available. If HIV negative/unknown, use dual HIV syphilis rapid test. If HIV positive ↗ 111. If syphilis positive ↗ 53. - If rapid tests unavailable: send syphilis serology (RPR). On request form, write: "If RPR titre 1:4 or less, do specific syphilis test on same specimen." If positive ↗ 53. - Follow positive syphilis results up: check mother has received all 3 treatment doses ↗ 53.
HIV	20, 26, 30, 34, 38 weeks and at delivery	- If HIV negative or status unknown, test for HIV ↗ 110. If patient refuses, offer at each visit, even in early labour. If test negative, consider need for PrEP ↗ 106. - If HIV positive, give routine HIV care and check that mother on ART. If not on ART, start ART same day ↗ 111.
Viral load (VL) if HIV positive	3 months on ART - At delivery	- If VL < 50, continue ART and repeat VL at delivery. - If VL ≥ 50, manage unsuppressed viral load ↗ 166.

Note: fill 'C#PMTCT' or 'C#DELIVERY' in code field on request form so VL not rejected.

Continue to advise and treat the pregnant patient → 163.

¹Symphysis-fundal height. ²Glucose in urine, BMI ≥ 32, age ≥ 40 years, previous diabetes in pregnancy, family history of diabetes, previous unexplained stillbirth, previous baby ≥ 4000g, polyhydramnios, SFH large for gestational age, Indian ethnicity.